

Data Dictionary Codebook

Access to medical oxygen scorecard (ATMOS) (PID: 411)

08-10-2026 14:28

#	Variable / Field Name	Field Label <i>Field Note</i>	Field Attributes (Field Type, Validation, Choices, Calculations, etc.)
Instrument: Form 1 (form_1)  Enabled as survey			
1	[record_id]	Record ID	text
2	[resp_countries]	Section Header: <i>RESPONDANT DETAILS</i> Which Member State or territory are you reporting for?	dropdown, Required
			1Afghanistan
			2Albania
			3Algeria
			4Andorra
			5Angola
			6Anguilla
			7Antigua and Barbuda
			8Argentina
			9Armenia
			10Australia
			11Austria
			12Azerbaijan
			13Bahamas
			14Bahrain
			15Bangladesh
			16Barbados
			17Belarus
			18Belgium
			19Belize

20	Benin
21	Bhutan
22	Bolivia (Plurinational State of)
23	Bosnia and Herzegovina
24	Botswana
25	Brazil
26	Brunei Darussalam
27	Bulgaria
28	Burkina Faso
29	Burundi
30	Cambodia
31	Cameroon
32	Canada
33	Cape Verde
34	Central African Republic
35	Chad
36	Chile
37	China
38	Colombia
39	Comoros
40	Cook Islands
41	Costa Rica
42	Croatia
43	Cuba
44	Cyprus
45	Czechia

46	Democratic People's Republic of Korea
47	Democratic Republic of the Congo
48	Denmark
49	Djibouti
50	Dominica
51	Dominican Republic
52	Ecuador
53	Egypt
54	El Salvador
55	Equatorial Guinea
56	Eritrea
57	Estonia
58	Eswatini
59	Ethiopia
60	Fiji
61	Finland
62	France
63	Gabon
64	Gambia
65	Georgia
66	Germany
67	Ghana
68	Greece
69	Grenada
70	Guatemala
71	Guinea

72	Guinea-Bissau
73	Guyana
74	Haiti
75	Honduras
76	Hungary
77	Iceland
78	India
79	Indonesia
80	Iran (Islamic Republic of)
81	Iraq
82	Ireland
83	Israel
84	Italy
85	Ivory Coast
86	Jamaica
87	Japan
88	Jordan
89	Kazakhstan
90	Kenya
91	Kiribati
92	Kuwait
93	Kyrgyzstan
94	Lao People's Democratic Republic
95	Latvia
96	Lebanon
97	Lesotho

98	Liberia
99	Libya
100	Lithuania
101	Luxembourg
102	Madagascar
103	Malawi
104	Malaysia
105	Maldives
106	Mali
107	Malta
108	Marshall Islands
109	Mauritania
110	Mauritius
111	Mexico
112	Micronesia (Federated States of)
113	Monaco
114	Mongolia
115	Montenegro
116	Morocco
117	Mozambique
118	Myanmar
119	Namibia
120	Nauru
121	Nepal
122	Netherlands
123	New Zealand

124	Nicaragua
125	Niger
126	Nigeria
127	Niue
128	North Macedonia
129	Norway
130	occupied Palestinian Territory including east Jerusalem
131	Oman
132	Pakistan
133	Palau
134	Panama
135	Papua New Guinea
136	Paraguay
137	Peru
138	Philippines
139	Poland
140	Portugal
141	Qatar
142	Republic of Korea
143	Republic of Moldova
144	Republic of the Congo
145	Romania
146	Russian Federation
147	Rwanda
148	Saint Kitts and Nevis

149	Saint Lucia
150	Saint Vincent and the Grenadines
151	Samoa
152	San Marino
153	Sao Tome and Principe
154	Saudi Arabia
155	Senegal
156	Serbia
157	Seychelles
158	Sierra Leone
159	Singapore
160	Slovakia
161	Slovenia
162	Solomon Islands
163	Somalia
164	South Africa
165	South Sudan
166	Spain
167	Sri Lanka
168	Sudan
169	Sudan (former)
170	Suriname
171	Sweden
172	Switzerland
173	Syrian Arab Republic
174	Tajikistan

			175	Tanzania
			176	Thailand
			177	The former state union Serbia and Montenegro
			178	Timor-Leste
			179	Togo
			180	Tonga
			181	Trinidad and Tobago
			182	Tunisia
			183	Turkey
			184	Turkmenistan
			185	Tuvalu
			186	Uganda
			187	Ukraine
			188	United Arab Emirates
			189	United Kingdom of Great Britain and Northern Ireland
			190	United States of America
			191	Uruguay
			192	Uzbekistan
			193	Vanuatu
			194	Venezuela (Bolivarian Republic of)
			195	Viet Nam
			196	Yemen
			197	Zambia
			198	Zimbabwe
3	[email]	Email address for respondent/contact	text	

4	[last_name]	Last/family name of respondent/contact	text						
5	[first_name]	First name of respondent/contact	text						
6	[resp_public]	Do you wish the results of the survey for your country/territory to be openly published by WHO?	radio, Required <table border="1"> <tr> <td>1</td> <td>Yes, data may be included in report to the Executive Board.</td> </tr> <tr> <td>0</td> <td>No, data are not to be shared or published in any form.</td> </tr> </table> Field Annotation: @DEFAULT = '1'	1	Yes, data may be included in report to the Executive Board.	0	No, data are not to be shared or published in any form.		
1	Yes, data may be included in report to the Executive Board.								
0	No, data are not to be shared or published in any form.								
7	[note_1]	Section Header: <i>1.1 National essential medicine lists</i> p, li, span, h3 { -webkit-hyphens: none !important; -ms-hyphens: none !important; hyphens: none !important; word-break: normal !important; overflow-wrap: normal !important; }Item"We include medical oxygen in national essential medicine lists (or an equivalent e.g. pharmacopoeia)."DetailsThe national level essential medicines list, or where this is not available, a national pharmacopoeia or formulary.Potential sources of evidence: Relevant pharmacopeia (if national, preferably to be updated by provisions in the International Pharmacopeia). Essential medicine lists, if available. Priority list of medical devices includes those needed for oxygen therapy and respiratory support, if available. <i>note</i>	descriptive						
8	[atmos11_score]	Self-assessment of national essential medicine lists	dropdown, Required <table border="1"> <tr> <td>1</td> <td>1 = Medical oxygen is not included in any essential medicine list.</td> </tr> <tr> <td>2</td> <td>2 = Medical oxygen is included within an essential medicine list, but is not considered essential, or is not aligned with international standards.</td> </tr> <tr> <td>3</td> <td>3 = Medical oxygen is included within a national essential medicine list, or equivalent.</td> </tr> </table>	1	1 = Medical oxygen is not included in any essential medicine list.	2	2 = Medical oxygen is included within an essential medicine list, but is not considered essential, or is not aligned with international standards.	3	3 = Medical oxygen is included within a national essential medicine list, or equivalent.
1	1 = Medical oxygen is not included in any essential medicine list.								
2	2 = Medical oxygen is included within an essential medicine list, but is not considered essential, or is not aligned with international standards.								
3	3 = Medical oxygen is included within a national essential medicine list, or equivalent.								
9	[atmos11_docu]	Do you wish to document details about this question?	radio, Required <table border="1"> <tr> <td>1</td> <td>Yes</td> </tr> <tr> <td>0</td> <td>No</td> </tr> </table>	1	Yes	0	No		
1	Yes								
0	No								

10	[atmos11_str] Show the field ONLY if: [atmos11_docu] = "1"	Current strengths and accomplishments <i>Write here if you wish to document strengths and accomplishments</i>	text						
11	[atmos11_plan] Show the field ONLY if: [atmos11_docu] = "1"	Challenges and plans to address them <i>Write here if you wish to document challenges and plans to address them</i>	text						
12	[note_2]	Section Header: <i>1.2 National strategic and operational health plans</i> p, li, span, h3 { -webkit-hyphens: none !important; -ms-hyphens: none !important; hyphens: none !important; word-break: normal !important; overflow-wrap: normal !important; }Item"We have included medical oxygen into national strategic and operational health plans (e.g. as a national oxygen strategy) which includes realistic costings and a financing plan."DetailsThis can include both incorporation into a broader strategic health planning document, or as a standalone oxygen roadmap/plan. If planning to develop a national plan, various tools are available to support. Realistic costing and financing should identify where funds are coming from, and be justified with data on oxygen need.Potential sources of evidence: A national-level document or strategy that covers the production, distribution and financing of medical oxygen, which covers the current time period (e.g. oxygen roadmap, oxygen scale-up plan, strategic health plan).	descriptive						
13	[atmos12_score]	Self-assessment for "national strategic and operational health plans"	dropdown, Required <table border="1"> <tr> <td>1</td> <td>1 = Medical oxygen is not currently covered by any national health strategy or plan.</td> </tr> <tr> <td>2</td> <td>2 = A strategy document exists, but it lacks core elements, such as costing or distribution planning.</td> </tr> <tr> <td>3</td> <td>3 = A fully costed national oxygen strategy for the current time period is available.</td> </tr> </table>	1	1 = Medical oxygen is not currently covered by any national health strategy or plan.	2	2 = A strategy document exists, but it lacks core elements, such as costing or distribution planning.	3	3 = A fully costed national oxygen strategy for the current time period is available.
1	1 = Medical oxygen is not currently covered by any national health strategy or plan.								
2	2 = A strategy document exists, but it lacks core elements, such as costing or distribution planning.								
3	3 = A fully costed national oxygen strategy for the current time period is available.								
14	[atmos12_docu]	Do you wish to document details about this question?	radio, Required <table border="1"> <tr> <td>1</td> <td>Yes</td> </tr> <tr> <td>0</td> <td>No</td> </tr> </table>	1	Yes	0	No		
1	Yes								
0	No								
15	[atmos12_str]	Current strengths and accomplishments	text						

	Show the field ONLY if: [atmos12_docu] = "1"	Write here if you wish to document strengths and accomplishments							
16	[atmos12_plan] Show the field ONLY if: [atmos12_docu] = "1"	Challenges and plans to address them <i>Write here if you wish to document challenges and plans to address them</i>	text						
17	[note_3]	Section Header: 1.3 Emergency and disaster preparedness and response plans p, li, span, h3 { -webkit-hyphens: none !important; -ms-hyphens: none !important; hyphens: none !important; word-break: normal !important; overflow-wrap: normal !important; }Item"We include medical oxygen in our approved national emergency and disaster preparedness and response plans and drills."DetailsEmergency and disaster response, in particular pandemic preparedness plans and drills, should allow for a range of scenarios, including a respiratory pandemic (such as influenza). However, oxygen need and planned response should be considered beyond respiratory pandemic scenarios.Potential sources of evidence: A pandemic preparedness plan which covers the current time period is in place and medical oxygen is included within this.	descriptive						
18	[atmos13_score]	Self-assessment of emergency and disaster preparedness and response plans	dropdown, Required <table><tr><td>1</td><td>1 = A pandemic preparedness plan is not in place, or if it is, medical oxygen is not included in the planning and response.</td></tr><tr><td>2</td><td>2 = A pandemic preparedness plan exists, but medical oxygen is insufficiently addressed (e.g. lack of surge planning).</td></tr><tr><td>3</td><td>3 = A pandemic preparedness plan exists, and medical oxygen is adequately considered across multiple scenarios.</td></tr></table>	1	1 = A pandemic preparedness plan is not in place, or if it is, medical oxygen is not included in the planning and response.	2	2 = A pandemic preparedness plan exists, but medical oxygen is insufficiently addressed (e.g. lack of surge planning).	3	3 = A pandemic preparedness plan exists, and medical oxygen is adequately considered across multiple scenarios.
1	1 = A pandemic preparedness plan is not in place, or if it is, medical oxygen is not included in the planning and response.								
2	2 = A pandemic preparedness plan exists, but medical oxygen is insufficiently addressed (e.g. lack of surge planning).								
3	3 = A pandemic preparedness plan exists, and medical oxygen is adequately considered across multiple scenarios.								
19	[atmos13_docu]	Do you wish to document details about this question?	radio, Required <table><tr><td>1</td><td>Yes</td></tr><tr><td>0</td><td>No</td></tr></table>	1	Yes	0	No		
1	Yes								
0	No								
20	[atmos13_str] Show the field ONLY if:	Current strengths and accomplishments <i>Write here if you wish to document strengths and accomplishments</i>	text						

	[atmos13_docu] = "1"								
21	[atmos13_plan] Show the field ONLY if: [atmos13_docu] = "1"	Challenges and plans to address them <i>Write here if you wish to document challenges and plans to address them</i>	text						
22	[note_4]	Section Header: 1.4 Coordination mechanisms p, li, span, h3 { -webkit-hyphens: none !important; -ms-hyphens: none !important; hyphens: none !important; word-break: normal !important; overflow-wrap: normal !important; }Item"We have coordination mechanisms in place across state and non-state actors to support effective partnerships, evidence use, advocacy and leveraged capabilities to increase and sustain oxygen access."DetailsState actors should be considered more broadly than the Ministry of Health. Non-state actors can include nongovernmental organizations, multilateral and bilateral organizations, private and industry and charities, civil society organizations and academia. Consider systems for integrating donations into the national oxygen system. Qualitative score assignment.Potential sources of evidence: Memoranda of understanding with partners. Oxygen taskforce has been convened. Guidance on managing medical donations; data sharing procedures. Prioritization based on needs assessments and local research.	descriptive						
23	[atmos14_score]	Self-assessment of coordination mechanisms	dropdown, Required <table><tr><td>1</td><td>1 = No clear mechanisms to support effective coordination of oxygen at a national level.</td></tr><tr><td>2</td><td>2 = Some coordination mechanisms in place, but their ability to impact oxygen access is limited (e.g. lack reach, partially active, lack leadership).</td></tr><tr><td>3</td><td>3 = Several currently active mechanisms exist to support coordination, with a person(s)/agency responsible for national oxygen coordination.</td></tr></table>	1	1 = No clear mechanisms to support effective coordination of oxygen at a national level.	2	2 = Some coordination mechanisms in place, but their ability to impact oxygen access is limited (e.g. lack reach, partially active, lack leadership).	3	3 = Several currently active mechanisms exist to support coordination, with a person(s)/agency responsible for national oxygen coordination.
1	1 = No clear mechanisms to support effective coordination of oxygen at a national level.								
2	2 = Some coordination mechanisms in place, but their ability to impact oxygen access is limited (e.g. lack reach, partially active, lack leadership).								
3	3 = Several currently active mechanisms exist to support coordination, with a person(s)/agency responsible for national oxygen coordination.								
24	[atmos14_docu]	Do you wish to document details about this question?	radio, Required <table><tr><td>1</td><td>Yes</td></tr><tr><td>0</td><td>No</td></tr></table>	1	Yes	0	No		
1	Yes								
0	No								

25	[atmos14_str] Show the field ONLY if: [atmos14_docu] = "1"	Current strengths and accomplishments <i>Write here if you wish to document strengths and accomplishments</i>	text						
26	[atmos14_plan] Show the field ONLY if: [atmos14_docu] = "1"	Challenges and plans to address them <i>Write here if you wish to document challenges and plans to address them</i>	text						
27	[note_5]	Section Header: <i>1.5 National health care infrastructure policy and oxygen integration</i> p, li, span, h3 { -webkit-hyphens: none !important; -ms-hyphens: none !important; hyphens: none !important; word-break: normal !important; overflow-wrap: normal !important; }Item"We have a national policy or planning document that considers the stepwise integration of medical oxygen and other medical gas systems into the construction of health care infrastructure."DetailsHealth facility infrastructure should consider the inclusion of medical oxygen systems (and other gases) in planning and development. Coordination with a health infrastructure unit (or equivalent) and inclusion in their policy and planning documentation will support holistic systems with all requisite infrastructure and hardware.Potential sources of evidence: National policy, regulations and standards for health care infrastructure.	descriptive						
28	[atmos15_score]	Self-assessment of national policy and oxygen integration	dropdown, Required <table border="1"> <tr> <td>1</td> <td>1 = Health infrastructure policy does not exist, or if it does, it makes no allowance for medical oxygen or other medical gas systems.</td> </tr> <tr> <td>2</td> <td>2 = Health infrastructure policy exists, but there are unaddressed issues of integration of medical oxygen and/or other medical gas systems.</td> </tr> <tr> <td>3</td> <td>3 = Comprehensive health infrastructure policy with integration of medical oxygen and other medical gas systems.</td> </tr> </table>	1	1 = Health infrastructure policy does not exist, or if it does, it makes no allowance for medical oxygen or other medical gas systems.	2	2 = Health infrastructure policy exists, but there are unaddressed issues of integration of medical oxygen and/or other medical gas systems.	3	3 = Comprehensive health infrastructure policy with integration of medical oxygen and other medical gas systems.
1	1 = Health infrastructure policy does not exist, or if it does, it makes no allowance for medical oxygen or other medical gas systems.								
2	2 = Health infrastructure policy exists, but there are unaddressed issues of integration of medical oxygen and/or other medical gas systems.								
3	3 = Comprehensive health infrastructure policy with integration of medical oxygen and other medical gas systems.								
29	[atmos15_docu]	Do you wish to document details about this question?	radio, Required <table border="1"> <tr> <td>1</td> <td>Yes</td> </tr> </table>	1	Yes				
1	Yes								

			<table><tr><td>0</td><td>No</td></tr></table>	0	No		
0	No						
30	[atmos15_str] Show the field ONLY if: [atmos15_docu] = "1"	Current strengths and accomplishments <i>Write here if you wish to document strengths and accomplishments</i>	text				
31	[atmos15_plan] Show the field ONLY if: [atmos15_docu] = "1"	Challenges and plans to address them <i>Write here if you wish to document challenges and plans to address them</i>	text				
32	[note_6]	Section Header: <i>2.1 Sufficient capacity in routine conditions</i> p, li, span, h3 { -webkit-hyphens: none !important; -ms-hyphens: none !important; hyphens: none !important; word-break: normal !important; overflow-wrap: normal !important; }Item"We can produce or acquire sufficient medical oxygen supply to meet need during routine conditions."DetailsSeasonal variation in oxygen need is included under "routine conditions". Ideally, data should be available from a nationally representative situational analysis and resource mapping, which can demonstrate both production capacity, effective distribution and oxygen need in the prior calendar year. Where data on these indicators are not available, a score is qualitatively assigned, considering any subnational/ad hoc data, e.g. stock-outs, consumption, hospitalizations. Information should be based on oxygen meeting specifications for medical application only (e.g. liquid oxygen for non-medical use should not be counted).Potential sources of evidence: Coverage and service readiness indicators available.	descriptive				
33	[atmos21_score]	Self-assessment of coordination mechanisms	dropdown, Required <table><tr><td>1</td><td>1 = Lack of systems in place to determine if oxygen supply has been sufficient, or multiple instances of oxygen being unavailable to meet needs in the prior calendar year.</td></tr><tr><td>2</td><td>2 = Systems in place to track and report medical oxygen production, distribution and need, but supply has inconsistently met need over the prior calendar year.</td></tr></table>	1	1 = Lack of systems in place to determine if oxygen supply has been sufficient, or multiple instances of oxygen being unavailable to meet needs in the prior calendar year.	2	2 = Systems in place to track and report medical oxygen production, distribution and need, but supply has inconsistently met need over the prior calendar year.
1	1 = Lack of systems in place to determine if oxygen supply has been sufficient, or multiple instances of oxygen being unavailable to meet needs in the prior calendar year.						
2	2 = Systems in place to track and report medical oxygen production, distribution and need, but supply has inconsistently met need over the prior calendar year.						

			3 3 = Systems are in place to track and report medical oxygen production, distribution and need, and that supply has met need over the prior calendar year.
34	[atmos21_docu]	Do you wish to document details about this question?	radio, Required 1 Yes 0 No
35	[atmos21_str] Show the field ONLY if: [atmos21_docu] = "1"	Current strengths and accomplishments <i>Write here if you wish to document strengths and accomplishments</i>	text
36	[atmos21_plan] Show the field ONLY if: [atmos21_docu] = "1"	Challenges and plans to address them <i>Write here if you wish to document challenges and plans to address them</i>	text
37	[note_7]	Section Header: <i>2.2 Capacity for surge demand</i> p, li, span, h3 { -webkit-hyphens: none !important; -ms-hyphens: none !important; hyphens: none !important; word-break: normal !important; overflow-wrap: normal !important; }Item"We can promptly increase medical oxygen supply to meet surges in demand in national- and subnational-level demands (e.g. outbreaks and disasters)."DetailsMajor surges are context and temporally dependent and definitions vary by context and scenario. Example scenarios include multiple overlapping seasonal viruses exceeding routine forecasts; a respiratory pandemic; a major trauma incident with increased critical care/surgery; natural disaster with damaged infrastructure. Qualitative score assignment, supported by guidelines and policy documents. Potential sources of evidence: Policies, plans and guidelines to consider include pandemic preparedness and response. Disaster response planning. Legal frameworks that allow for temporary conversion of industrial to medical oxygen. Mechanisms for emergency release of government funds.	descriptive
38	[atmos22_score]	Self-assessment of capacity for surge demand	dropdown, Required 1 1 = Guidelines and policies to support surge capacity are not available or multiple instances of

			oxygen being unavailable to meet needs in the prior calendar year. 2 2 = Medical oxygen is included in plans and guidelines, but for limited scenarios or with insufficient coverage for surge capacity, or supply has inconsistently met need over the prior calendar year. 3 3 = Medical oxygen is included in plans and guidelines for various different scenarios that would require surge capacity, and supply has met need over the prior calendar year.
39	[atmos22_docu]	Do you wish to document details about this question?	radio, Required 1 Yes 0 No
40	[atmos22_str] Show the field ONLY if: [atmos22_docu] = "1"	Current strengths and accomplishments <i>Write here if you wish to document strengths and accomplishments</i>	text
41	[atmos22_plan] Show the field ONLY if: [atmos22_docu] = "1"	Challenges and plans to address them <i>Write here if you wish to document challenges and plans to address them</i>	text
42	[note_8]	Section Header: <i>2.3 Quantification of need</i> p, li, span, h3 { -webkit-hyphens: none !important; -ms-hyphens: none !important; hyphens: none !important; word-break: normal !important; overflow-wrap: normal !important; }Item" We have an up-to-date quantification of national medical oxygen need and this is linked to production and procurement."Details This currently focuses on a national-level approach to forecasting medical oxygen need, but subnational variation should be considered when building this capacity. A system for quantifying oxygen need, allowing for changes in production capacity and seasonal variation, should be available. The score is qualitatively assigned, considering performance of forecasting and subnational application. Potential sources of evidence: When assessing the need-gap, it should be done holistically (i.e. consider all related medical devices required	descriptive

		for medical oxygen therapy, including those for diagnosis and treatment of hypoxaemia and all medical oxygen system requirements for production, storage and distribution of medical oxygen). It should also consider the functional status of all equipment. Coverage and service readiness indicators available.							
43	[<i>atmos23_score</i>]	Self-assessment of quantification of need	dropdown, Required <table><tr><td>1</td><td>1 = Quantification of medical oxygen need is not conducted.</td></tr><tr><td>2</td><td>2 = Quantification of medical oxygen need is conducted, but is out of date or does not link to production and procurement capacity.</td></tr><tr><td>3</td><td>3 = Quantification of medical oxygen need is up to date, and includes subnational variation.</td></tr></table>	1	1 = Quantification of medical oxygen need is not conducted.	2	2 = Quantification of medical oxygen need is conducted, but is out of date or does not link to production and procurement capacity.	3	3 = Quantification of medical oxygen need is up to date, and includes subnational variation.
1	1 = Quantification of medical oxygen need is not conducted.								
2	2 = Quantification of medical oxygen need is conducted, but is out of date or does not link to production and procurement capacity.								
3	3 = Quantification of medical oxygen need is up to date, and includes subnational variation.								
44	[<i>atmos23_docu</i>]	Do you wish to document details about this question?	radio, Required <table><tr><td>1</td><td>Yes</td></tr><tr><td>0</td><td>No</td></tr></table>	1	Yes	0	No		
1	Yes								
0	No								
45	[<i>atmos23_str</i>] Show the field ONLY if: [atmos23_docu] = "1"	Current strengths and accomplishments <i>Write here if you wish to document strengths and accomplishments</i>	text						
46	[<i>atmos23_plan</i>] Show the field ONLY if: [atmos23_docu] = "1"	Challenges and plans to address them <i>Write here if you wish to document challenges and plans to address them</i>	text						
47	[<i>note_9</i>]	Section Header: <i>2.4 Guidance and contracts</i> p, li, span, h3 { -webkit-hyphens: none !important; -ms-hyphens: none !important; hyphens: none !important; word-break: normal !important; overflow-wrap: normal !important; }Item"We have guidance documents and contracts to connect production (e.g. gas companies, distributors) and end-user organizations (e.g. hospitals, district health boards) at national and subnational levels."DetailsBeyond contracts there should also be legal mechanisms that ensure the obligations of contracts are met. Guidelines for negotiating contracts exist at the national level, and all levels of the health system are covered by this. Evidence of	descriptive						

		service-level agreements can be used where available, and should consider maintenance, training and distribution models.Potential sources of evidence: Sustainable procurement guidelines. Service-level agreements with inclusion of spare parts and continuous training. Mechanisms for the beneficiary facilities to access the contracts, the legal support and the supplier's assistance when applicable. Service readiness indicator available. Availability of and access to local, regional or international specialized services for maintenance of existing medical oxygen systems.							
48	[atmos24_score]	Self-assessment of guidance and contracts	dropdown, Required <table><tr><td>1</td><td>1 = Guidelines for negotiating contracts do not exist at the national level.</td></tr><tr><td>2</td><td>2 = Guidelines for negotiating contracts exist, but they do not consider different health system level needs.</td></tr><tr><td>3</td><td>3 = Guidelines for negotiating contracts exist at the national level, and all levels of the health system are covered by this.</td></tr></table>	1	1 = Guidelines for negotiating contracts do not exist at the national level.	2	2 = Guidelines for negotiating contracts exist, but they do not consider different health system level needs.	3	3 = Guidelines for negotiating contracts exist at the national level, and all levels of the health system are covered by this.
1	1 = Guidelines for negotiating contracts do not exist at the national level.								
2	2 = Guidelines for negotiating contracts exist, but they do not consider different health system level needs.								
3	3 = Guidelines for negotiating contracts exist at the national level, and all levels of the health system are covered by this.								
49	[atmos24_docu]	Do you wish to document details about this question?	radio, Required <table><tr><td>1</td><td>Yes</td></tr><tr><td>0</td><td>No</td></tr></table>	1	Yes	0	No		
1	Yes								
0	No								
50	[atmos24_str] Show the field ONLY if: [atmos24_docu] = "1"	Current strengths and accomplishments <i>Write here if you wish to document strengths and accomplishments</i>	text						
51	[atmos24_plan] Show the field ONLY if: [atmos24_docu] = "1"	Challenges and plans to address them <i>Write here if you wish to document challenges and plans to address them</i>	text						
52	[note_10]	Section Header: <i>2.5 Regulatory processes</i> p, li, span, h3 { -webkit-hyphens: none !important; -ms-hyphens: none !important; hyphens: none !important; word-break: normal !important; overflow-wrap: normal !important; }Item"We have regulatory processes for ensuring medical oxygen, respiratory care medical devices and associated equipment meet quality and safety	descriptive						

		standards and accountability mechanisms for those responsible for supply."DetailsThis includes both commercial medical oxygen, industrial oxygen and donations of medical oxygen and of equipment. Regulatory instruments and standards can be demonstrated at relevant national agencies (e.g. drug and medical device authorities) in the first instance. Evidence that they are functional and being implemented in the next local level. Facility-level processes should also be considered when assigning the score qualitatively.Potential sources of evidence: Established good manufacturing practices for medical oxygen. National and local regulations, policies and technical specifications for medicines and medical devices manufacturing, distribution and use. Mechanism for reporting adverse events or medical incidents (e.g. toxicity related to medical oxygen). Service readiness indicator.							
53	[atmos25_score]	Self-assessment of regulatory processes	dropdown, Required <table><tr><td>1</td><td>1 = Regulatory standards for medical oxygen, respiratory care medical devices and associated equipment are not present.</td></tr><tr><td>2</td><td>2 = Regulatory instruments and standards for medical oxygen, respiratory care medical devices and associated equipment exist, but they are insufficient or lack oversight.</td></tr><tr><td>3</td><td>3 = Regulatory instruments and standards for medical oxygen, respiratory care medical devices and associated equipment exist and there is active oversight in the supply chain.</td></tr></table>	1	1 = Regulatory standards for medical oxygen, respiratory care medical devices and associated equipment are not present.	2	2 = Regulatory instruments and standards for medical oxygen, respiratory care medical devices and associated equipment exist, but they are insufficient or lack oversight.	3	3 = Regulatory instruments and standards for medical oxygen, respiratory care medical devices and associated equipment exist and there is active oversight in the supply chain.
1	1 = Regulatory standards for medical oxygen, respiratory care medical devices and associated equipment are not present.								
2	2 = Regulatory instruments and standards for medical oxygen, respiratory care medical devices and associated equipment exist, but they are insufficient or lack oversight.								
3	3 = Regulatory instruments and standards for medical oxygen, respiratory care medical devices and associated equipment exist and there is active oversight in the supply chain.								
54	[atmos25_docu]	Do you wish to document details about this question?	radio, Required <table><tr><td>1</td><td>Yes</td></tr><tr><td>0</td><td>No</td></tr></table>	1	Yes	0	No		
1	Yes								
0	No								
55	[atmos25_str] Show the field ONLY if: [atmos25_docu] = "1"	Current strengths and accomplishments <i>Write here if you wish to document strengths and accomplishments</i>	text						
56	[atmos25_plan] Show the field ONLY if:	Challenges and plans to address them <i>Write here if you wish to document challenges and plans to address them</i>	text						

	[atmos25_docu] = "1"								
57	[note_11]	Section Header: 2.6 Safe distribution p, li, span, h3 { -webkit-hyphens: none !important; -ms-hyphens: none !important; hyphens: none !important; word-break: normal !important; overflow-wrap: normal !important; }Item" We have mechanisms for safe distribution of medical oxygen that cover all facilities, to ensure minimal gaps in availability at the point of care for all patients in need of medical oxygen." DetailsThis includes plans for transport and storage at facilities and should include both logistics and safety considerations. When assessing distribution, it should include remote and less accessible geographical areas in all seasons of the year. Potential sources of evidence: Established good distribution practices for medical oxygen. Occupational and environmental health standards covering medical oxygen distribution. Data from sources such as incidence reports and stock-outs of medical oxygen to healthcare facilities at all levels, including both rural and urban. Service readiness indicator available.	descriptive						
58	[atmos26_score]	Self-assessment of safe distribution	dropdown, Required <table><tr><td>1</td><td>1 = Medical oxygen distribution mechanisms are not established, or are not covered by environmental and occupational safety standards.</td></tr><tr><td>2</td><td>2 = Occupational and environmental health cover medical oxygen distribution, but there are gaps in distribution systems.</td></tr><tr><td>3</td><td>3 = Occupational and environmental health cover medical oxygen distribution and distribution systems cover all facilities.</td></tr></table>	1	1 = Medical oxygen distribution mechanisms are not established, or are not covered by environmental and occupational safety standards.	2	2 = Occupational and environmental health cover medical oxygen distribution, but there are gaps in distribution systems.	3	3 = Occupational and environmental health cover medical oxygen distribution and distribution systems cover all facilities.
1	1 = Medical oxygen distribution mechanisms are not established, or are not covered by environmental and occupational safety standards.								
2	2 = Occupational and environmental health cover medical oxygen distribution, but there are gaps in distribution systems.								
3	3 = Occupational and environmental health cover medical oxygen distribution and distribution systems cover all facilities.								
59	[atmos26_docu]	Do you wish to document details about this question?	radio, Required <table><tr><td>1</td><td>Yes</td></tr><tr><td>0</td><td>No</td></tr></table>	1	Yes	0	No		
1	Yes								
0	No								
60	[atmos26_str] Show the field ONLY if: [atmos26_docu] = "1"	Current strengths and accomplishments Write here if you wish to document strengths and accomplishments	text						

61	[atmos26_plan] Show the field ONLY if: [atmos26_docu] = "1"	Challenges and plans to address them <i>Write here if you wish to document challenges and plans to address them</i>	text						
62	[note_12]	Section Header: 3.1 Budget allocations p, li, span, h3 { -webkit-hyphens: none !important; -ms-hyphens: none !important; hyphens: none !important; word-break: normal !important; overflow-wrap: normal !important; }Item"We have budget allocations for the key priorities within national medical oxygen plans and are able to access and release funds in a timely way."DetailsKey considerations are how different funding sources are being used, what areas are being prioritized, differentiating between capital and recurrent costs across production, distribution and delivery, and whether there are accountability mechanisms. National, subnational, district or facility budget documents (depending on the way oxygen is procured) should be the basis of the scoring, with both the amount allocated to oxygen systems and the amount spent in the last full budget year considered.Potential sources of evidence: National health budget.	descriptive						
63	[atmos31_score]	Self-assessment of budget allocations	dropdown, Required <table><tr><td>1</td><td>1 = Medical oxygen systems were not included in the annual health budget, or funds allocated for oxygen were not realized.</td></tr><tr><td>2</td><td>2 = Medical oxygen systems were only partially included in the annual health budget (i.e. key components of the oxygen plan were missing), and funds were not fully realized.</td></tr><tr><td>3</td><td>3 = Medical oxygen systems were fully covered in the annual health budget, and funds were largely realized.</td></tr></table>	1	1 = Medical oxygen systems were not included in the annual health budget, or funds allocated for oxygen were not realized.	2	2 = Medical oxygen systems were only partially included in the annual health budget (i.e. key components of the oxygen plan were missing), and funds were not fully realized.	3	3 = Medical oxygen systems were fully covered in the annual health budget, and funds were largely realized.
1	1 = Medical oxygen systems were not included in the annual health budget, or funds allocated for oxygen were not realized.								
2	2 = Medical oxygen systems were only partially included in the annual health budget (i.e. key components of the oxygen plan were missing), and funds were not fully realized.								
3	3 = Medical oxygen systems were fully covered in the annual health budget, and funds were largely realized.								
64	[atmos31_docu]	Do you wish to document details about this question?	radio, Required <table><tr><td>1</td><td>Yes</td></tr><tr><td>0</td><td>No</td></tr></table>	1	Yes	0	No		
1	Yes								
0	No								
65	[atmos31_str]	Current strengths and accomplishments <i>Write here if you wish to document strengths and accomplishments</i>	text						

	Show the field ONLY if: [atmos31_docu] = "1"								
66	[atmos31_plan] Show the field ONLY if: [atmos31_docu] = "1"	Challenges and plans to address them <i>Write here if you wish to document challenges and plans to address them</i>	text						
67	[note_13]	Section Header: 3.2 Procurement and tendering processes p, li, span, h3 { -webkit-hyphens: none !important; -ms-hyphens: none !important; hyphens: none !important; word-break: normal !important; overflow-wrap: normal !important; }Item"Our procurement and tendering processes for medical oxygen supplies, services and devices are transparent and reflect a competitive market price."DetailsNational-level guidance on public tenders should be available. This should consider both the processes used for tendering oxygen services, and how they are applied to ensure a transparent and competitive market. Other evidence that should be qualitatively discussed are presence of freedom of information laws for government contracts; price transparency from medical oxygen companies; presence and application of laws that prevent monopolies.Potential sources of evidence: Mechanisms to track suppliers' quality performance and fulfilment of contracts. Sustainable procurement policies. Use of technical specifications which allow for transparent competition and local manufacturing.	descriptive						
68	[atmos32_score]	Self-assessment of procurement and tendering processes	dropdown, Required <table><tr><td>1</td><td>1 = National policies on public tenders do not exist or are inadequate.</td></tr><tr><td>2</td><td>2 = National policies on public tenders exist, but supportive mechanisms to ensure a competitive market are not available or functional.</td></tr><tr><td>3</td><td>3 = National policies on public tenders exist, and mechanisms to ensure they are applied in a transparent and competitive way are functional.</td></tr></table>	1	1 = National policies on public tenders do not exist or are inadequate.	2	2 = National policies on public tenders exist, but supportive mechanisms to ensure a competitive market are not available or functional.	3	3 = National policies on public tenders exist, and mechanisms to ensure they are applied in a transparent and competitive way are functional.
1	1 = National policies on public tenders do not exist or are inadequate.								
2	2 = National policies on public tenders exist, but supportive mechanisms to ensure a competitive market are not available or functional.								
3	3 = National policies on public tenders exist, and mechanisms to ensure they are applied in a transparent and competitive way are functional.								
69	[atmos32_docu]	Do you wish to document details about this question?	radio, Required <table><tr><td>1</td><td>Yes</td></tr></table>	1	Yes				
1	Yes								

			0 No						
70	[atmos32_str] Show the field ONLY if: [atmos32_docu] = "1"	Current strengths and accomplishments <i>Write here if you wish to document strengths and accomplishments</i>	text						
71	[atmos32_plan] Show the field ONLY if: [atmos32_docu] = "1"	Challenges and plans to address them <i>Write here if you wish to document challenges and plans to address them</i>	text						
72	[note_14]	Section Header: <i>3.3 Affordability to providers</i> p, li, span, h3 { -webkit-hyphens: none !important; -ms-hyphens: none !important; hyphens: none !important; word-break: normal !important; overflow-wrap: normal !important; } Item "We have mechanisms in place that ensure the provision of medical oxygen services at the point of care is affordable to providers and uses appropriate technologies (devices, spare parts, consumables, servicing)." Details This focuses on how affordable it is for healthcare providers to access an appropriate mix of oxygen technologies for patient care. Qualitative score assignment, considering the presence of a national formulary for medical devices and drugs, which has an appropriate equipment mix and variations in price points for different use cases. The use of health technology assessments is considered. Potential sources of evidence: Medical devices that are affordable, appropriate, accessible, safe and effective as part of efforts to achieve universal health coverage. Coverage and quality of care indicators available.	descriptive						
73	[atmos33_score]	Self-assessment of affordability to providers	dropdown, Required <table border="1"> <tbody> <tr> <td>1</td> <td>1 = National formulary does not exist for either drugs or medical devices.</td> </tr> <tr> <td>2</td> <td>2 = National formulary exists for either drugs or medical devices, but not for both.</td> </tr> <tr> <td>3</td> <td>3 = National formulary exists for both drugs and medical devices.</td> </tr> </tbody> </table>	1	1 = National formulary does not exist for either drugs or medical devices.	2	2 = National formulary exists for either drugs or medical devices, but not for both.	3	3 = National formulary exists for both drugs and medical devices.
1	1 = National formulary does not exist for either drugs or medical devices.								
2	2 = National formulary exists for either drugs or medical devices, but not for both.								
3	3 = National formulary exists for both drugs and medical devices.								
74	[atmos33_docu]	Do you wish to document details about this question?	radio, Required 1 Yes						

			<table><tr><td>0</td><td>No</td></tr></table>	0	No				
0	No								
75	[atmos33_str] Show the field ONLY if: [atmos33_docu] = "1"	Current strengths and accomplishments <i>Write here if you wish to document strengths and accomplishments</i>	text						
76	[atmos33_plan] Show the field ONLY if: [atmos33_docu] = "1"	Challenges and plans to address them <i>Write here if you wish to document challenges and plans to address them</i>	text						
77	[note_15]	Section Header: <i>3.4 Mechanisms for affordability</i> p, li, span, h3 { -webkit-hyphens: none !important; -ms-hyphens: none !important; hyphens: none !important; word-break: normal !important; overflow-wrap: normal !important; }Item"We have mechanisms in place that ensure oxygen is affordable to all patients who need it (e.g. insurance, financial assistance, subsidies)."DetailsThis is patient centred and should consider universal health coverage policies and financial protections. Evidence of policies that include universal health coverage and financial protections (including insurance) should be qualitatively discussed. Where available, data on out-of-pocket and catastrophic health expenditure should be considered. Affordability throughout the course of treatment, including for home oxygen therapy, should be considered when applicable.Potential sources of evidence: Evidence of insurance, financial assistance or subsidy schemes.	descriptive						
78	[atmos34_score]	Self-assessment of mechanisms for affordability	dropdown, Required <table><tr><td>1</td><td>1 = Oxygen services are not covered by national insurance or financial protection policies, and can result in catastrophic health expenditure.</td></tr><tr><td>2</td><td>2 = Oxygen services can incur out-of-pocket costs to patients, which are only partially covered or only for parts of the population.</td></tr><tr><td>3</td><td>3 = National health system is free at the point of care, or comprehensive financial protections are in place and include oxygen services.</td></tr></table>	1	1 = Oxygen services are not covered by national insurance or financial protection policies, and can result in catastrophic health expenditure.	2	2 = Oxygen services can incur out-of-pocket costs to patients, which are only partially covered or only for parts of the population.	3	3 = National health system is free at the point of care, or comprehensive financial protections are in place and include oxygen services.
1	1 = Oxygen services are not covered by national insurance or financial protection policies, and can result in catastrophic health expenditure.								
2	2 = Oxygen services can incur out-of-pocket costs to patients, which are only partially covered or only for parts of the population.								
3	3 = National health system is free at the point of care, or comprehensive financial protections are in place and include oxygen services.								

79	[atmos34_docu]	Do you wish to document details about this question?	radio, Required <table><tr><td>1</td><td>Yes</td></tr><tr><td>0</td><td>No</td></tr></table>	1	Yes	0	No
1	Yes						
0	No						
80	[atmos34_str] Show the field ONLY if: [atmos34_docu] = "1"	Current strengths and accomplishments <i>Write here if you wish to document strengths and accomplishments</i>	text				
81	[atmos34_plan] Show the field ONLY if: [atmos34_docu] = "1"	Challenges and plans to address them <i>Write here if you wish to document challenges and plans to address them</i>	text				
82	[note_16]	Section Header: <i>4.1 Trained healthcare workers</i> p, li, span, h3 { -webkit-hyphens: none !important; -ms-hyphens: none !important; hyphens: none !important; word-break: normal !important; overflow-wrap: normal !important; }Item"We have sufficient trained and equipped healthcare workers to provide medical oxygen services safely and effectively to patients."DetailsSufficient refers to the existing WHO standard for health workforce of five health care workers per 1000 population (which is a general standard not specific to oxygen), and should further consider how these health workers are distributed in the health system to ensure oxygen access. While this focuses on a national level, addressing subnational variation needs to be considered.Potential sources of evidence: Evidence of the number of currently working healthcare workers at a national level. Oxygen and pulse oximetry is included as a core competency in the curriculum for organizations which confer clinical degrees and qualifications. Coverage, quality-of-care and service readiness indicators available.	descriptive				
83	[atmos41_score]	Self-assessment of training of healthcare workers	dropdown, Required <table><tr><td>1</td><td>1 = Below five health care workers per 1000 AND training in oxygen and oximetry are not routinely, or ever, conducted, or data on healthcare worker density are not available.</td></tr><tr><td>2</td><td>2 = Below five health care workers per 1000 population OR training in oxygen and oximetry are</td></tr></table>	1	1 = Below five health care workers per 1000 AND training in oxygen and oximetry are not routinely, or ever, conducted, or data on healthcare worker density are not available.	2	2 = Below five health care workers per 1000 population OR training in oxygen and oximetry are
1	1 = Below five health care workers per 1000 AND training in oxygen and oximetry are not routinely, or ever, conducted, or data on healthcare worker density are not available.						
2	2 = Below five health care workers per 1000 population OR training in oxygen and oximetry are						

			not routinely conducted.
			3 3 = A minimum of five health care workers per 1000 population, and these are distributed effectively at subnational and health system levels, with training on oxygen and pulse oximetry.
84	[atmos41_docu]	Do you wish to document details about this question? <i>Write here if you wish to document strengths and accomplishments</i>	radio, Required 1 Yes 0 No
85	[atmos41_str] Show the field ONLY if: [atmos41_docu] = "1"	Current strengths and accomplishments <i>Write here if you wish to document strengths and accomplishments</i>	text
86	[atmos41_plan] Show the field ONLY if: [atmos41_docu] = "1"	Challenges and plans to address them <i>Write here if you wish to document challenges and plans to address them</i>	text
87	[note_17]	Section Header: <i>4.2 Trained engineers and technicians</i> p, li, span, h3 { -webkit-hyphens: none !important; -ms-hyphens: none !important; hyphens: none !important; word-break: normal !important; overflow-wrap: normal !important; }Item"We have sufficient competent biomedical engineers/technicians, with access to appropriate equipment and supplies, to manage medical oxygen technology safely and effectively."DetailsSufficient biomedical engineers does not currently have a WHO standard; national-level requirements, addressing subnational variation, need to be considered, as well as systems for different levels of the health system and oxygen technology types. The presence of a capacity-building strategy should also be considered.Potential sources of evidence: Evidence on the number of trained and currently working biomedical engineers and technicians at a national level. Facility-level coverage can also be used to evidence sufficient coverage.	descriptive
88	[atmos42_score]	Self-assessment of competent engineers and technicians	dropdown, Required 1 1 = Data on biomedical engineer density are not available AND there is considerable subnational variation and inadequate coverage in biomedical

			engineer workforce. No strategic plan for increasing workforce capacity. 2 2 = Data are available on biomedical engineers nationally but there remains considerable subnational variation and inadequate coverage in the biomedical engineer workforce. Presence of a strategic plan for increasing workforce capacity. 3 3 = Data are available on biomedical engineers nationally and subnationally, and these are distributed effectively at subnational and health system level. A strategic plan for increasing workforce capacity exists.				
89	[atmos42_docu]	Do you wish to document details about this question?	radio, Required <table border="1"> <tr> <td>1</td> <td>Yes</td> </tr> <tr> <td>0</td> <td>No</td> </tr> </table>	1	Yes	0	No
1	Yes						
0	No						
90	[atmos42_str] Show the field ONLY if: [atmos42_docu] = "1"	Current strengths and accomplishments <i>Write here if you wish to document strengths and accomplishments</i>	text				
91	[atmos42_plan] Show the field ONLY if: [atmos42_docu] = "1"	Challenges and plans to address them <i>Write here if you wish to document challenges and plans to address them</i>	text				
92	[note_18]	Section Header: <i>4.3 Monitoring and evaluation</i> p, li, span, h3 { -webkit-hyphens: none !important; -ms-hyphens: none !important; hyphens: none !important; word-break: normal !important; overflow-wrap: normal !important; }Item"We have indicators for monitoring and evaluating medical oxygen systems, including financing, supply, need, patient access and quality of care (safety and appropriateness) at national, subnational and health facility levels."DetailsThis item considers both the systems for monitoring oxygen systems, but also the evaluation of oxygen systems based on monitoring data. Evidence of a monitoring and evaluation framework, which includes all dimensions of medical oxygen systems and covers the current time period, should be available. The quality of data, extent to which data have been	descriptive				

		adopted (e.g. indicators included within health management information systems) and application of this for evaluation (e.g. annual reports) should be qualitatively discussed.Potential sources of evidence: Monitoring and evaluation framework. Quality management systems applied along the supply chain of medical oxygen.							
93	[atmos43_score]	Self-assessment of monitoring and evaluation	dropdown, Required <table><tr><td>1</td><td>1 = Medical oxygen systems are not currently included in a national health monitoring and evaluation framework.</td></tr><tr><td>2</td><td>2 = Monitoring and evaluation framework exists, but it does not cover all elements of the medical oxygen system, or has not been translated into implementation.</td></tr><tr><td>3</td><td>3 = Monitoring and evaluation framework exists which covers all aspects of the medical oxygen system, and which has been adopted into routine data systems and is used in annual evaluation reports.</td></tr></table>	1	1 = Medical oxygen systems are not currently included in a national health monitoring and evaluation framework.	2	2 = Monitoring and evaluation framework exists, but it does not cover all elements of the medical oxygen system, or has not been translated into implementation.	3	3 = Monitoring and evaluation framework exists which covers all aspects of the medical oxygen system, and which has been adopted into routine data systems and is used in annual evaluation reports.
1	1 = Medical oxygen systems are not currently included in a national health monitoring and evaluation framework.								
2	2 = Monitoring and evaluation framework exists, but it does not cover all elements of the medical oxygen system, or has not been translated into implementation.								
3	3 = Monitoring and evaluation framework exists which covers all aspects of the medical oxygen system, and which has been adopted into routine data systems and is used in annual evaluation reports.								
94	[atmos43_docu]	Do you wish to document details about this question?	radio, Required <table><tr><td>1</td><td>Yes</td></tr><tr><td>0</td><td>No</td></tr></table>	1	Yes	0	No		
1	Yes								
0	No								
95	[atmos43_str] Show the field ONLY if: [atmos43_docu] = "1"	Current strengths and accomplishments <i>Write here if you wish to document strengths and accomplishments</i>	text						
96	[atmos43_plan] Show the field ONLY if: [atmos43_docu] = "1"	Challenges and plans to address them <i>Write here if you wish to document challenges and plans to address them</i>	text						
97	[note_19]	Section Header: 4.4 Public health information p, li, span, h3 { -webkit-hyphens: none !important; -ms-hyphens: none !important; hyphens: none !important; word-break: normal !important; overflow-wrap: normal !important; }Item"We have reliable public health information being communicated to the	descriptive						

		population about medical oxygen services available."DetailsThe types of materials and their purpose will be context dependent and should respond to local needs - it is important to consider what is relevant in your context. Examples could include information targeted at the general population to promote uptake of referrals for oxygen treatment; information for health care workers to support effective patient communication; tackling misinformation. Evidence of public health materials that have been checked/updated within the prior 12 months should be provided and scored through qualitative discussion considering their relevance to context and timeliness.Potential sources of evidence: Web page with information. Awareness campaigns. Posters and materials for health care workers.							
98	[<i>atmos44_score</i>]	Self-assessment of public health information	dropdown, Required <table><tr><td>1</td><td>1 = Public health materials on medical oxygen are not available through the Ministry of Health or public health agency.</td></tr><tr><td>2</td><td>2 = Public health materials are available, but are out of date, not tailored to the specific needs of the country, or are inaccessible to part of the population.</td></tr><tr><td>3</td><td>3 = Up-to-date public health materials available (in multiple languages where appropriate) and tailored to the different needs of communities, patients and providers.</td></tr></table>	1	1 = Public health materials on medical oxygen are not available through the Ministry of Health or public health agency.	2	2 = Public health materials are available, but are out of date, not tailored to the specific needs of the country, or are inaccessible to part of the population.	3	3 = Up-to-date public health materials available (in multiple languages where appropriate) and tailored to the different needs of communities, patients and providers.
1	1 = Public health materials on medical oxygen are not available through the Ministry of Health or public health agency.								
2	2 = Public health materials are available, but are out of date, not tailored to the specific needs of the country, or are inaccessible to part of the population.								
3	3 = Up-to-date public health materials available (in multiple languages where appropriate) and tailored to the different needs of communities, patients and providers.								
99	[<i>atmos44_docu</i>]	Do you wish to document details about this question?	radio, Required <table><tr><td>1</td><td>Yes</td></tr><tr><td>0</td><td>No</td></tr></table>	1	Yes	0	No		
1	Yes								
0	No								
100	[<i>atmos44_str</i>] Show the field ONLY if: [atmos44_docu] = "1"	Current strengths and accomplishments <i>Write here if you wish to document strengths and accomplishments</i>	text						

101	[atmos44_plan] Show the field ONLY if: [atmos44_docu] = "1"	Challenges and plans to address them <i>Write here if you wish to document challenges and plans to address them</i>	text						
102	[note_20]	Section Header: 4.5 Appropriate use p, li, span, h3 { -webkit-hyphens: none !important; -ms-hyphens: none !important; hyphens: none !important; word-break: normal !important; overflow-wrap: normal !important; }Item"We have systems in place to ensure appropriate use of oxygen among those who need it."DetailsSystems refer to guidelines that guide clinical care, and accountability mechanisms that ensure quality of care (including safe use of medical oxygen). Appropriate use of medical oxygen refers to both the safe delivery of medical oxygen to the patient, and the diagnosis and monitoring of patients using pulse oximetry. Where possible, the score should be data driven. Without data, evidence of systems should be used and discussed qualitatively.Potential sources of evidence: National clinical guidelines for medical oxygen use (adapted to different cadres and health system levels). Standard operating procedures for use and maintenance of medical equipment and other devices. Facility audits for oxygen production and distribution equipment; up-to-date inventory of functional equipment. Patient surveys. Adverse event data for patients (e.g. neonatal retinopathy). Adverse event data for systems or equipment.	descriptive						
103	[atmos45_score]	Self-assessment of appropriate use	dropdown, Required <table><tr><td>1</td><td>1 = No formal systems or data available to demonstrate appropriate use of oxygen.</td></tr><tr><td>2</td><td>2 = Guidelines exist for the clinical use of medical oxygen, but systems/data for ensuring they are safely and effectively used are lacking.</td></tr><tr><td>3</td><td>3 = Guidelines exist for clinical use of medical oxygen, and systems for ensuring these are applied safely and effectively are in place.</td></tr></table>	1	1 = No formal systems or data available to demonstrate appropriate use of oxygen.	2	2 = Guidelines exist for the clinical use of medical oxygen, but systems/data for ensuring they are safely and effectively used are lacking.	3	3 = Guidelines exist for clinical use of medical oxygen, and systems for ensuring these are applied safely and effectively are in place.
1	1 = No formal systems or data available to demonstrate appropriate use of oxygen.								
2	2 = Guidelines exist for the clinical use of medical oxygen, but systems/data for ensuring they are safely and effectively used are lacking.								
3	3 = Guidelines exist for clinical use of medical oxygen, and systems for ensuring these are applied safely and effectively are in place.								
104	[atmos45_docu]	Do you wish to document details about this question?	radio, Required <table><tr><td>1</td><td>Yes</td></tr></table>	1	Yes				
1	Yes								

			<table><tr><td>0</td><td>No</td></tr></table>	0	No				
0	No								
105	[atmos45_str] Show the field ONLY if: [atmos45_docu] = "1"	Current strengths and accomplishments <i>Write here if you wish to document strengths and accomplishments</i>	text						
106	[atmos45_plan] Show the field ONLY if: [atmos45_docu] = "1"	Challenges and plans to address them <i>Write here if you wish to document challenges and plans to address them</i>	text						
107	[note_21]	Section Header: <i>4.6 Research and innovation</i> p, li, span, h3 { -webkit-hyphens: none !important; -ms-hyphens: none !important; hyphens: none !important; word-break: normal !important; overflow-wrap: normal !important; }Item"We promote research and innovation, including translational research, to improve access, quality and safety of affordable medical oxygen and related respiratory care devices and diagnostics."DetailsResearch forms the basis for evidence-informed policy and health care. Innovation provides new and improved solutions to deliver these policies and care. Both can be accelerated by national research agendas, active coordination and translational collaboration.Potential sources of evidence: National research agenda. Initiatives at local higher education institutes.	descriptive						
108	[atmos46_score]	Self-assessment of coordination mechanisms	dropdown, Required <table><tr><td>1</td><td>1 = Minimal or no oxygen research is undertaken, and oxygen is not included in relevant research agendas.</td></tr><tr><td>2</td><td>2 = Some oxygen research is undertaken, but there is limited engagement of researchers/innovators, or a lack of research prioritization.</td></tr><tr><td>3</td><td>3 = Oxygen is included in relevant research agendas, with active engagement of researchers/innovators.</td></tr></table>	1	1 = Minimal or no oxygen research is undertaken, and oxygen is not included in relevant research agendas.	2	2 = Some oxygen research is undertaken, but there is limited engagement of researchers/innovators, or a lack of research prioritization.	3	3 = Oxygen is included in relevant research agendas, with active engagement of researchers/innovators.
1	1 = Minimal or no oxygen research is undertaken, and oxygen is not included in relevant research agendas.								
2	2 = Some oxygen research is undertaken, but there is limited engagement of researchers/innovators, or a lack of research prioritization.								
3	3 = Oxygen is included in relevant research agendas, with active engagement of researchers/innovators.								

109	[atmos46_docu]	Do you wish to document details about this question?	radio, Required <table><tr><td>1</td><td>Yes</td></tr><tr><td>0</td><td>No</td></tr></table>	1	Yes	0	No		
1	Yes								
0	No								
110	[atmos46_str] Show the field ONLY if: [atmos46_docu] = "1"	Current strengths and accomplishments <i>Write here if you wish to document strengths and accomplishments</i>	text						
111	[atmos46_plan] Show the field ONLY if: [atmos46_docu] = "1"	Challenges and plans to address them <i>Write here if you wish to document challenges and plans to address them</i>	text						
112	[atmos_score]	Section Header: <i>OVERALL SCORE</i> ATMOS overall score: <i>This score represents the sum of your responses to all self-assessment questions in this survey.</i>	calc Calculation: [atmos11_score]+[atmos12_score]+ [atmos13_score]+[atmos14_score]+[atmos15_score]+ [atmos21_score]+[atmos22_score]+[atmos23_score]+ [atmos24_score]+[atmos25_score]+[atmos26_score]+ [atmos31_score]+[atmos32_score]+[atmos33_score]+ [atmos34_score]+ [atmos41_score]+[atmos42_score]+ [atmos43_score]+[atmos44_score]+[atmos45_score]+ [atmos46_score]						
113	[form_1_complete]	Section Header: <i>Form Status</i> Complete?	dropdown <table><tr><td>0</td><td>Incomplete</td></tr><tr><td>1</td><td>Unverified</td></tr><tr><td>2</td><td>Complete</td></tr></table>	0	Incomplete	1	Unverified	2	Complete
0	Incomplete								
1	Unverified								
2	Complete								